

Wardle Editorial Operating System (WEOS)

Part VIII - Institutional Editorial Governance

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1. Purpose

The Wardle Editorial Operating System governs clinical knowledge on behalf of learners, educators, reviewers, and institutions.

Clinical knowledge does not become trustworthy merely because it has been written, generated, reviewed, or technically published.

It becomes institutionally trusted when qualified people exercise defined authority through transparent, accountable, and reproducible editorial governance.

The purpose of Institutional Editorial Governance is to define how responsibility, authority, accountability, and oversight operate throughout WEOS.

This chapter establishes:

- who contributes editorial knowledge;
- who may evaluate it;
- who may make editorial decisions;
- how authority is assigned;
- how disagreements are resolved;
- how conflicts of interest are managed;
- how evidence and high-risk content receive appropriate oversight;
- how publication authority is exercised;
- how governance remains accountable;
- how editorial governance scales from individual editors to institutional and multi-institutional structures.

Institutional Editorial Governance does not define user-interface permissions or specific organisational job titles.

It defines the enduring authority model that every implementation and editorial organisation must preserve.

2. Philosophy of Institutional Governance

WEOS treats editorial governance as an institutional responsibility rather than an individual activity.

An individual editor may possess expertise.

A reviewer may possess experience.

A specialty board may possess collective authority.

However, expertise alone does not establish institutional trust.

Institutional trust emerges when expertise is exercised through:

- defined responsibilities;
- appropriate scope of authority;
- transparent assessment;
- explicit decisions;
- preserved rationale;
- evidence-informed judgement;
- accountable publication;
- continuing oversight.

Governance should protect the editorial system from:

- unreviewed clinical assertions;
- informal approval;
- authority based solely on system access;
- publication without accountability;
- concentration of incompatible responsibilities;
- hidden conflicts of interest;
- inconsistent specialty standards;
- unresolved reviewer disagreement;
- unsupported exceptions;
- loss of institutional memory.

Institutional governance strengthens clinical judgement by placing it within a durable framework of responsibility.

3. Institutional Trust

Institutional trust represents the confidence that an organisation places in governed clinical knowledge.

It is not equivalent to confidence in a particular author, editor, artificial-intelligence system, or external source.

Institutional trust is created through the combined effect of:

- stable editorial identity;
- appropriate clinical scope;
- evidence;
- qualified review;
- educational evaluation;
- governance decisions;
- dependency assessment;

- publication authority;
- maintenance;
- preserved history.

Institutional trust belongs to the governed artifact and its approved version.

It does not transfer automatically from the reputation of its creator.

Institutional trust is earned through governance rather than inherited from origin.

4. Governance Scope

Institutional Editorial Governance operates across complementary levels.

4.1 Artifact Governance

Artifact Governance evaluates an individual editorial artifact.

It asks whether the artifact is clinically defensible, educationally appropriate, evidence-supported, correctly scoped, and eligible to progress.

4.2 Diagnosis Governance

Diagnosis Governance evaluates the complete editorial foundation of a diagnosis, including identity, metadata, educational intent, cases, education, relationships, evidence, dependencies, and maintenance.

4.3 Specialty Governance

Specialty Governance evaluates knowledge within the clinical standards and educational priorities of a specialty or subspecialty.

4.4 Curriculum Governance

Curriculum Governance evaluates whether independently governed artifacts collectively form an appropriate programme of learning.

4.5 Institutional Governance

Institutional Governance oversees authority, policies, quality, evidence, publication controls, maintenance, auditability, and organisational accountability.

An artifact may be clinically acceptable while its diagnosis remains incomplete.

A diagnosis may be well developed while specialty-level inconsistencies remain unresolved.

Institutional governance ensures that trust is evaluated at every relevant level.

5. Editorial Authority

Editorial authority is the recognised ability to make a governed decision within a defined scope.

Authority should reflect:

- professional competence;
- editorial responsibility;
- artifact risk;
- decision consequence;
- clinical scope;
- educational scope;
- institutional mandate.

Authority must not be inferred solely from technical access.

The ability to change a database record, apply a migration, or operate an administrative interface does not confer clinical or publication authority.

Editorial authority should always answer:

- Who may make this decision?
- Which artifact or domain does the authority cover?
- Which conditions limit the authority?
- Which decisions require additional review?
- Who remains accountable for the outcome?
- How is the decision recorded?

6. Editorial Roles

WEOS recognises editorial roles according to responsibility rather than software permission.

6.1 Contributor

A Contributor introduces candidate knowledge, identified errors, references, feedback, or maintenance concerns.

Contribution establishes provenance but not authority.

6.2 Editorial Author

An Editorial Author develops candidate artifacts according to approved educational intent and responds to review findings.

Authorship does not automatically confer approval authority.

6.3 Clinical Reviewer

A Clinical Reviewer evaluates factual correctness, diagnostic interpretation, investigations, management, safety, severity, population applicability, and risk of harmful misunderstanding.

6.4 Educational Reviewer

An Educational Reviewer evaluates learning purpose, reasoning progression, clue sequencing, learner level, misconception prevention, educational usefulness, and curriculum alignment.

6.5 Knowledge Governance Reviewer

A Knowledge Governance Reviewer evaluates structured relationships, graph facts, reasoning paths, differential mappings, contradiction, duplication, provenance, and downstream consequences.

6.6 Evidence Reviewer

An Evidence Reviewer evaluates source authority, recency, applicability, methodological quality, claim alignment, and conflicting evidence.

6.7 Specialty Reviewer

A Specialty Reviewer provides deeper authority for specialist diagnostic nuance, advanced investigations, high-risk management, uncommon disease, or evolving guidance.

6.8 Senior Editor

A Senior Editor resolves complex decisions, coordinates multidisciplinary review, supervises specialty governance, and authorises high-impact editorial outcomes within their mandate.

6.9 Publication Authority

The Publication Authority determines whether governed knowledge may become learner-facing or institutionally operational.

6.10 Editorial Operations Lead

The Editorial Operations Lead assigns work, monitors queues, coordinates reviewers, tracks maintenance, and escalates operational risk without silently exercising clinical authority.

6.11 System Administrator

A System Administrator manages access, migrations, restoration, and technical operation.

Administrative capability does not create editorial authority.

7. Role Qualification and Scope

Editorial authority should be based upon demonstrable suitability for the decision being made.

Suitability may include:

- clinical training;
- specialty experience;
- educational experience;
- editorial competence;
- familiarity with WEOS standards;

- evidence-appraisal competence;
- knowledge-governance competence;
- institutional appointment;
- continuing participation in quality review.

Qualification should be proportionate to risk.

Where an artifact requires several forms of authority, governance should combine them rather than assume one reviewer possesses every necessary perspective.

8. Editorial Ownership

Every actively governed artifact should have identifiable editorial ownership.

Ownership establishes responsibility for continued stewardship rather than unrestricted decision authority.

Ownership may exist at artifact, diagnosis, specialty, curriculum, publication, and maintenance levels.

Ownership should answer:

- Who ensures work progresses?
- Who monitors unresolved concerns?
- Who coordinates required reviewers?
- Who responds when evidence changes?
- Who ensures published knowledge remains maintained?

Ownership must remain transferable without losing governance history.

9. Assignment of Editorial Work

Editorial work should be assigned according to competence, artifact type, specialty, risk, learner impact, urgency, independence, and availability.

Assignment creates responsibility to review or act.

It does not predetermine the decision.

Reviewers should remain free to approve, reject, request revision, request evidence, defer, escalate, declare insufficient expertise, or declare a conflict of interest.

10. Separation of Duties

Certain responsibilities should remain separate where combining them would weaken governance.

Separation may require that:

- an author does not provide the only approval of their own work;
- technical application does not substitute for editorial approval;
- publication is authorised separately from generation;
- high-risk clinical content receives independent review;
- disputed decisions are adjudicated independently;
- emergency actions receive retrospective review;
- migrations affecting governed knowledge are authorised before execution.

Where small teams require overlapping roles, governance should compensate through disclosure, additional review, preserved rationale, audit, and restricted publication authority.

11. Assessment and Decision Authority

Editorial assessments and editorial decisions are distinct.

An assessment interprets quality or state.

A decision changes or confirms governed standing.

Decision authority should be proportionate to consequence.

11.1 Ordinary Editorial Decisions

Examples include routine wording revision, duplicate-candidate rejection, low-risk Learning Goal approval, or straightforward alias resolution.

11.2 Elevated Editorial Decisions

Examples include Clinical Case approval, Teaching Relationship activation, Diagnosis Education approval, or governed-artifact supersession.

11.3 High-Risk Editorial Decisions

Examples include emergency management, drug doses, serious-diagnosis exclusion claims, pregnancy or paediatric treatment, and cross-specialty guidance.

11.4 Institutional Decisions

Examples include publication, withdrawal, governance exceptions, diagnosis retirement, and adoption of institutional standards.

12. Editorial Disagreement

Disagreement is an expected feature of responsible clinical governance.

WEOS should not conceal disagreement through silent editing or informal compromise.

A disagreement record should identify:

- disputed issue;
- affected artifact;
- competing interpretations;
- evidence considered;
- clinical and educational consequences;
- reviewers involved;
- unresolved questions;
- required escalation.

The objective is not forced consensus.

The objective is a defensible institutional decision.

13. Escalation and Adjudication

Escalation occurs when ordinary review cannot safely or appropriately resolve an issue.

It may be required because of reviewer disagreement, insufficient expertise, high risk, contradictory evidence, cross-specialty impact, unresolved conflict, or publication urgency.

Adjudication should preserve:

- competing positions;
- evidence considered;
- adjudicating authority;
- final rationale;
- conditions;
- future review obligations.

Adjudication should not erase earlier disagreement.

14. Conflict of Interest

A conflict of interest exists when judgement may be influenced, or reasonably perceived to be influenced, by interests outside the editorial merits of the artifact.

Possible responses include:

- disclosure;
- restricted participation;
- independent review;
- removal from final authority;
- reassignment;
- additional approval.

Undeclared conflicts weaken institutional trust even where the decision is clinically correct.

15. Evidence Oversight

Clinical evidence supports editorial decisions but does not replace them.

Evidence Oversight should ensure that:

- claims are linked to appropriate support;
- sources apply to the stated population and context;
- evidence remains sufficiently current;
- conflicting evidence is represented honestly;
- high-risk claims receive proportionate scrutiny;
- retracted or superseded evidence is identified;
- evidence changes trigger maintenance.

A reference may be credible but not applicable.

Evidence contributes to institutional confidence without independently determining publication.

16. Governance of High-Risk Knowledge

High-risk knowledge may include:

- drug doses;
- emergency treatment;
- procedural guidance;
- contraindications;
- antimicrobial recommendations;
- paediatric and pregnancy management;
- critical-care thresholds;
- diagnostic exclusion statements;
- escalation criteria;
- scoring-system cut-offs;
- guideline-dependent management.

High-risk governance may require stronger evidence, specialty review, independent review, dual approval, explicit scope, stronger validation, shorter maintenance intervals, and defined emergency correction pathways.

Risk should be determined by the consequence of error rather than artifact type alone.

17. Publication Authority

Publication represents an institutional decision to permit governed knowledge to influence learners or institutional activity.

Publication authority must remain separate from authorship, generation, validation, technical deployment, scheduling, and operational availability.

A Publication Authority may publish, schedule, defer, restrict, withdraw, supersede, or decline publication.

The absence of an automated blocker does not constitute publication approval.

18. Withdrawal, Supersession, and Retirement

Published knowledge may require withdrawal, supersession, or retirement.

Withdrawal removes knowledge from active exposure because continued use is inappropriate.

Supersession replaces it with a preferred successor while preserving history.

Retirement ends active editorial use when the artifact or diagnosis no longer serves an appropriate purpose.

Removal from active use must not erase institutional history.

19. Emergency Editorial Governance

Emergency action may be required where learner-facing knowledge presents a credible and significant risk.

Possible actions include immediate withdrawal, temporary suppression, scheduling suspension, disabling playability, restricting generation, or urgent correction.

Every emergency action should preserve authority, risk, time, action, expected duration, and required follow-up.

Urgency may shorten the pathway.

It must not remove accountability.

20. Governance Committees and Editorial Boards

As WEOS grows, governance may be distributed across specialty boards, clinical-safety panels, educational-quality committees, knowledge-governance boards, publication committees, curriculum committees, and evidence groups.

Every governance body should possess:

- defined scope;

- membership requirements;
- decision authority;
- conflict rules;
- preserved decisions;
- escalation pathways;
- review responsibilities.

Committees should not become symbolic approval layers.

21. Specialty Governance

Specialty Governance may define specialty ownership, terminology, reviewer requirements, high-risk categories, evidence expectations, maintenance priorities, and curriculum expectations.

Specialty decisions should remain visible to the wider ecosystem.

Cross-specialty knowledge should support joint review rather than arbitrary ownership.

22. Curriculum Governance

Curriculum Governance evaluates whether the knowledge ecosystem provides coherent learning.

It should consider diagnosis representation, Learning Goal coverage, important mimics, case diversity, learner progression, population representation, specialty balance, and maintenance.

Curriculum governance should not be reduced to counting cases.

Curriculum confidence emerges from educational coherence rather than content volume.

23. Relationship to Editorial Intelligence

Editorial Intelligence may identify blockers, recommend reviewers, detect conflicts, surface evidence deficiencies, prioritise maintenance, calculate readiness, and recommend escalation.

Editorial Intelligence improves governance awareness.

It does not possess governance authority.

A recommendation to approve is not approval.

A readiness result is not publication.

24. Relationship to Artificial Intelligence

Artificial Intelligence may generate, summarise, compare, detect, propose, and prepare context.

It must not independently establish clinical authority, accept evidence, adjudicate disagreement, approve high-risk knowledge, publish, withdraw, or grant exceptions.

AI may assist the decision process.

Human editorial governance remains responsible for the decision.

25. Governance Records

Every material governance action should produce a durable record preserving artifact identity, version, question, actor, authority, evidence, findings, rationale, decision, state change, unresolved concerns, and required follow-up.

A status without rationale is insufficient governance.

A decision without an identifiable decision-maker is insufficient governance.

A changed artifact without version history is insufficient governance.

26. Governance Exceptions

Exceptions should remain rare, explicit, limited, and reviewable.

An exception should identify the requirement, reason, authority, scope, duration, risk, compensating controls, follow-up, and expiry.

Temporary necessity should not become permanent policy through repeated informal use.

27. Governance Quality Assurance

Governance quality may be assessed through completeness of review records, consistency of decisions, evidence sufficiency, disagreement, post-publication correction, unresolved decision age, unauthorised action, emergency-action frequency, overdue maintenance, and separation-of-duty compliance.

WEOS should improve the system producing decisions rather than only correcting individual defects.

28. External Review and Accreditation

External review should be able to determine which standards govern knowledge, who possesses authority, how decisions are recorded, how evidence is assessed, how publication occurs, how errors are corrected, and how AI contributions are governed.

External endorsement may strengthen confidence.

It does not replace internal accountability.

29. Governance Across Institutional Scale

The same principles should remain valid as WEOS grows.



Distributed authority should remain transparent and accountable.

Knowledge approved by one institution should not automatically become approved by every participating institution.

30. Governance Continuity

Governance must remain resilient when people, organisations, and technologies change.

Continuity requires stable identities, transferable ownership, documented authority, preserved decisions, succession, access continuity, restoration capability, maintained standards, and periodic revalidation.

Institutional knowledge should not depend upon the memory of one editor.

31. Principles of Institutional Editorial Governance

Authority Must Be Explicit. Authority should not be inferred solely from authorship, seniority, reputation, or technical access.

Responsibility Must Be Identifiable. Every material decision should have an accountable human decision-maker.

Expertise Must Be Appropriately Scoped. Reviewers should act within their competence and mandate.

Approval Must Apply to a Version. Approval must not apply ambiguously to mutable knowledge.

Publication Requires Separate Authority. Approved knowledge must not become learner-facing without an explicit Publication Decision.

High-Risk Knowledge Requires Stronger Governance. Governance intensity should reflect the consequence of error.

Disagreement Must Remain Visible. Material disagreement should be preserved through resolution.

Conflicts Must Be Managed Transparently. Undisclosed conflicts weaken confidence.

Evidence Supports but Does Not Govern. Sources inform decisions but do not independently create trust.

Technical Capability Does Not Confer Clinical Authority. Administrative and engineering access remain distinct from editorial authority.

Governance Must Remain Auditable. Decisions require context, rationale, responsibility, and history.

Institutional Memory Must Outlive Individuals. Governance should remain durable through personnel and technology change.

32. Relationship to the Wider WEOS Architecture

The Editorial Domain Model defines what is governed.

Editorial Capabilities define what work must be possible.

Editorial Lifecycles define how confidence changes.

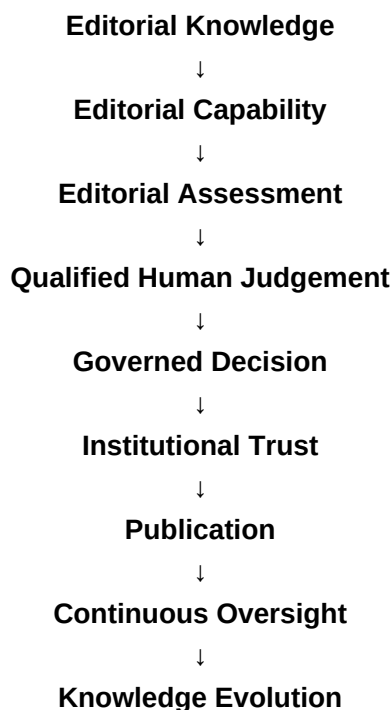
Knowledge Production defines how candidate knowledge enters.

Editorial Intelligence interprets what requires attention.

The Editorial Workspace supports completion of work.

Institutional Editorial Governance defines who may exercise authority, how decisions become trusted, and how accountability is preserved.

The Evolution of the Editorial Knowledge Ecosystem describes how these structures grow.



Closing Principle

Clinical knowledge does not become trustworthy because it was created by an expert, generated by an advanced system, supported by a reference, or made available through software.

It becomes trustworthy when appropriate expertise is exercised through transparent, accountable, evidence-informed, and continuously maintained institutional governance.

People contribute knowledge.

Reviewers evaluate knowledge.

Editorial Intelligence supports judgement.

Governance establishes authority.

Institutions assume responsibility.

Publication expresses trust.

Accountability preserves that trust over time.